

761 : Master.Aayush Sonbawane (2m 1d, Male) - 7498477562
Date : 25-Jul-2026

Impression: Chief Complaint:

Ayush was brought with a complaint of inability to move the right hip for the past 8 days.

History: According to the parents, Ayush was completely well until 8 days ago, after which he started crying whenever his right hip was touched or moved. Since then, he has been reluctant to move the right lower limb. Initial evaluation and treatment were carried out at another center, where an ultrasonography was reported as normal. However, blood investigations revealed anemia and a markedly elevated C-reactive protein (CRP) of 74 mg/L, raising concern for an underlying inflammatory or infective process.

Clinical Examination:

On examination:

- * Suspension test is positive.* Right hip range of motion is markedly painful.* The child is unable to fully extend the right knee because of pain around the hip.
- * Upper limb examination is normal.* Spine examination is normal.* No distal neurovascular deficit is appreciated.

Investigations:

Plain radiographs of the pelvis and hips:* No periosteal reaction is seen.* Mild widening of the right hip joint space is noted, raising suspicion of an intra-articular effusion/collection.

Previous Blood Investigations:* Anemia present.* CRP: 74 mg/L (elevated).

Clinical Impression:

The clinical presentation, elevated inflammatory markers, and radiographic findings are highly suggestive of an infective process involving the right hip, with septic arthritis of the hip being the primary consideration. Associated osteomyelitis cannot be excluded at this stage.

Discussion with Parents:

I explained to the parents, in simple and understandable language, that Ayush's history, examination findings, elevated CRP, and X-ray appearance are strongly suggestive of an infection around the right hip joint. Although the previous ultrasonography was reported as normal, the current clinical findings warrant repeat targeted imaging, as fluid within the joint or a periosteal collection may have developed or may have been missed earlier. I explained that septic arthritis of the hip is an orthopedic emergency, and prompt diagnosis is essential to prevent damage to the hip joint and growth plate. The natural history, prognosis, need for urgent evaluation, and possible treatment options, including surgical drainage if an effusion is confirmed, were discussed in detail in a language the parents could understand. All questions were answered to the parents' satisfaction.

Plan: Dear Dr. Ajinkya sir,

- * Urgent ultrasonography of the right hip to evaluate for:
- * Hip joint effusion.
- * Periosteal or adjacent soft tissue collection.
- * Repeat inflammatory markers as clinically indicated.

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* Review immediately with the ultrasonography findings.

* Further management, including possible aspiration or surgical drainage with intravenous antibiotics, will be determined based on the imaging findings and overall clinical assessment.

The parents understood the provisional diagnosis, the urgency of further evaluation, and the proposed management plan. All questions were answered, and reassurance was provided.



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